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HARVARD UNIVERSITY

Education

Harvard University
Ph.D. in Population Health Sciences (Health Systems and Economics), 2019–2024
University of Nottingham
M.Sc., Economics and Econometrics, Distinction, 2014
University of Nottingham
B.Sc., Economics, First Class Honors, 2013

Fields

Health Economics
Industrial Organization
Development Economics

References

Mark Shepard
Harvard Kennedy School
mark_shepard@hks.harvard.edu

Luca Maini
Harvard Medical School
maini@hcp.med.harvard.edu

Winnie Yip
Harvard T.H. Chan School of Public Health
wyip@hsph.harvard.edu

Fellowships & Awards

Fulbright Scholarship, 2019

University of Nottingham Dean's List, 2010–2013; University of Nottingham M.Sc. Scholarship, 2014

Teaching

Research for Health Economics, Harvard Kennedy School, Teaching Fellow for Prof. Mark Shepard, 2025, 2026

Econometrics for Health Policy, Harvard T.H. Chan School of Public Health, Teaching Fellow for Prof. Terence Cheng, 2022, 2023

Comparative Health Systems I and II, Harvard T.H. Chan School of Public Health, Teaching Fellow for Prof. Winnie Yip, 2022

Introduction to Macroeconomics and Microeconomics, University of Nottingham, Teaching Fellow for Profs. Saumik Paul and Wing Leong Teo, 2012

Employment

Harvard Kennedy School, Taubman Center for State and Local Government — Postdoctoral Research Fellow, 2026–present

Harvard T.H. Chan School of Public Health, Department of Global Health and Population — Postdoctoral Research Fellow, 2024–2026

Harvard T.H. Chan School of Public Health — Visiting Scientist, 2015–2018

World Bank — Consultant (Economic Analyst, International Bank for Reconstruction and Development), 2015, 2019

United Nations Development Programme — Consultant (Program Evaluation, housing projects, Malaysia), 2014

Research

Research Assistant, University of Nottingham — Professor Saumik Paul, 2013

Research Assistant, Harvard T.H. Chan School of Public Health — Professor Winnie Yip, 2019–2020

Job Market Paper

"Subsidizing Quality for the Poor: Equity and Competition in Health Insurance Markets"

For many essential goods and services, the poor are often priced out of higher-quality options. Governments respond by incrementally subsidizing these options to ensure affordability for the poor. This paper studies the trade-offs of incremental subsidies in the Massachusetts health insurance exchange, where the state paid part of the price gap between higher-quality (broad-network) plans and lower-quality plans for low-income enrollees. Using the removal of these subsidies in 2017, I show that the policy afforded low-income enrollees access to broad-network plans but weakened price competition among insurers. I estimate that 88 cents of every incremental subsidy dollar became insurer profits despite increasing the poor's enrollment in the broad-plan. The policy cannot be rationalized by a planner who maximizes consumer surplus net of public spending, and instead suggests a planner who places additional value on low-income enrollment in broad-network plans. A counterfactual fixed subsidy to enrollees choosing a broad-network plan yields higher enrollment at the same public cost, with lower insurer profits. Overall, for a planner who values equity in quality, subsidizing quality directly is optimal relative to subsidizing incremental prices.

Publications

"Hospital Allocation and Politicians: Evidence from Malaysia." World Development, 2026, 207: 107495.

Equitable access to healthcare requires efficient allocation of health infrastructure, yet politics may distort these decisions. This paper studies how politicians distort public hospital allocations in postcolonial Malaysia from 1959 to 2013. Malaysia first allocates hospital funding to districts, then selects specific locations within funded districts for construction. Using six decades of data on hospital construction and elections, I find that constituencies represented by deputy ministers are significantly more likely to receive new hospitals, but only at the location selection stage within districts. District-level funding decisions effectively direct resources to districts far from existing hospitals. I provide suggestive evidence that measurable guidelines for funding decisions exist but not for construction site selection. These results suggest that defining clear guidelines in hospital allocation can limit political distortions.

"Consumer Choice and Public–Private Providers: The Role of Perceived Prices" (with Wei Aun Yap and Winnie Yip). Health Economics, 2022, 31(9): 1898–1925.

Governments often encourage health service providers to improve quality of care and reduce prices through competition. The efficacy of competition hinges on the assumption that consumers demand high quality care at low prices for any given health condition. In this paper, we examine this assumption by investigating the role of perceived price and quality on consumer choice for four different health conditions across public and private providers. We use a nationally representative survey in Malaysia to elicit respondents' perception on prices and quality, and their preferred choice of provider. We estimate a mixed logit model and show that consumers value different dimensions of quality depending on the health condition. Furthermore, increasing perceived prices for private providers reduces demand for minor, more frequent health conditions such as flu fever or cough, but increases demand for more complex, severe conditions such as coronary artery bypass graft. These findings provide empirical support for price regulation which differentiates the severity of underlying health conditions.

Working Papers

"Strategic Auto-Enrollment and Health Insurance Market Design" (with Mark Shepard and Myles Wagner)

Default rules are typically viewed as demand-side tools to steer behavior and increase take-up among passive consumers. We argue that in markets with supply-side competition, defaults also operate as a *market design* policy, shaping firms' incentives to compete on price and quality. We

develop a framework for analyzing optimal "strategic defaults" that highlights a central tradeoff between two effects: a matching effect (assigning passive consumers to plans that fit their needs) and a competition effect (the strength of pricing and product-design incentives induced by the assignment rule). We apply this framework to automatic enrollment in Massachusetts' pre-ACA subsidized health insurance exchange, where roughly one-third of below-poverty enrollees are passively assigned across competing plans with differentiated provider networks. Leveraging quasi-random assignment, we document economically meaningful matching effects: enrollees defaulted into plans covering nearby hospitals use more physician care and are substantially less likely to switch out. We then develop a structural model of active and passive enrollees that quantifies the matching-competition tradeoff and characterizes optimal default rules, including assignments that reward plan quality and exploit consumer-plan match characteristics.

"Competing Complements in Public–Private Hospital Markets" (Under review, American Economic Journal: Economic Policy)

Public hospitals in many countries provide subsidized care that competes with private hospitals, while training physicians who later join the private sector. I study this tension using the staggered construction of public hospitals in Malaysia from 1996 to 2013. A new public hospital increases private hospital entrants by 36 percent. This crowd-in requires both labor-supply spillovers from public hospitals and districts large enough to support private entry. Reallocating public hospitals to larger districts would raise crowded-in private capacity by roughly a third but reduce equity in access. These findings show public hospitals trade off equity-driven allocation against total healthcare capacity.

"Does Word-of-Mouth Lead to Better Care? Evidence from Odisha, India" (with Terence C. Cheng and Bijetri Bose) (Submitted, The Journal of Development Studies)

Word-of-mouth (WOM) recommendations from family and friends can substitute for absent quality certification and weak regulatory oversight. Whether such informal information improves or distorts patient decisions remains unclear. We study this question using original linked household and facility survey data from Odisha, India, covering 3,090 patient visits across 373 villages. Patients who choose providers based on recommendations spend more out-of-pocket on drugs and transportation. Yet, this additional spending does not translate into better outcomes: WOM is not associated with higher perceived care quality, and WOM users are significantly less likely to return to the same provider. To understand these patterns, we estimate an attentive logit model that separately identifies consideration and preference stages of provider choice. We find that WOM narrows attention to a smaller set of providers that are not higher-quality. Counterfactual simulations show that removing WOM-induced attention distortions does not significantly improve the quality of providers patients choose, while raising awareness towards other providers yields modest gains. These findings suggest that informal networks cannot reliably substitute for formal quality information systems in fragmented healthcare markets.

Health Policy Papers

McIntyre A, Stone B, **Lim KS**, Shepard M, Sommers BD. "Massachusetts Marketplace Disenrollment Following Enhanced Subsidy Expiration." *JAMA*, forthcoming.

Seminars & Conferences

BU/Harvard/MIT Health Economics Seminar, 2026
Northeast Universities Development Consortium (NEUDC), Tufts University, 2025
American Society of Health Economists (ASHEcon) Annual Conference, 2023–2026

Academic Service

Referee: Social Science & Medicine; Health Systems & Reform

Languages

English (native), Malay (fluent), Mandarin (basic)